



Pharm*Achieve*

ETHICS

Qualifying Exam Preparatory Course

COMPETENCIES COVERED IN THIS PRESENTATION...

Providing Care:
Clinical Care

Providing Care:
Distribution

Knowledge
& Expertise

Communication
& Collaboration

Leadership &
Stewardship

Professionalism

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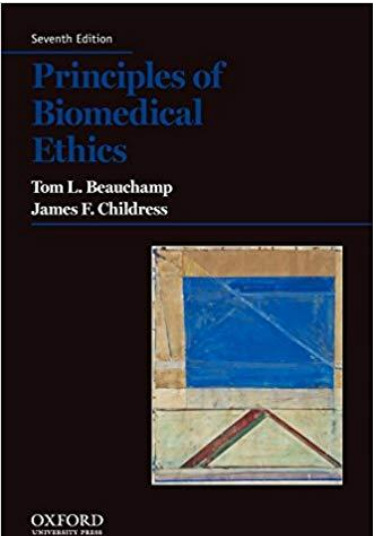
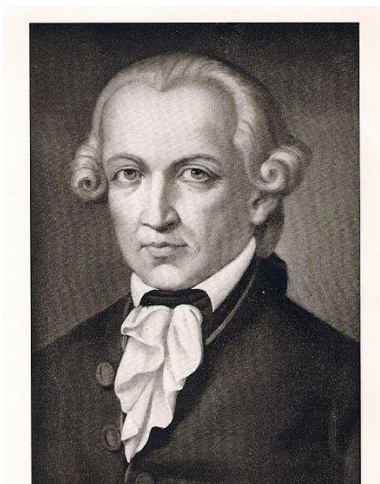
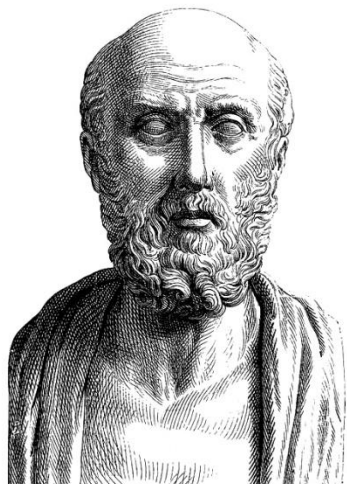
DEFINITIONS

	Morals	vs.	Ethics
Source	Internal: Derived from an individual's conscience, values, and personal beliefs (often influenced by family, culture, and religion)		External: Established by social systems, professions, or organizations. Examples include a code of conduct for a profession or societal laws
Scope	Personal and Subjective: can vary significantly from person to person		Group-based and Objective: provide a uniform set of rules that apply to every member of the group, aiming for a consistent standard of behavior
Key Idea	Your inner sense of right/wrong		Shared, generally-accepted principles on how one should behave

CONSENT: A **patient's permission** to undergo a treatment or research, or to collect, store or disclose, or use information

RESEARCH: Collection of **patient data**, through interventions that go beyond those necessary for the individual's immediate well-being

SOME ORIGINS OF BIOETHICAL PRINCIPLES



Time

Hippocrates 460 BC
Non-Maleficence
Beneficence
Fidelity

Immanuel Kant (1724-1804)
Autonomy

Nuremberg Code (NC) ~1945
Declaration of Helsinki (DOH) (1975)

Human experiments must be for greater good (NC, DOH)
Voluntary participation (NC, DOH)
Informed decisions (DOH)

Beauchamp and Childress (1978)
Autonomy
Beneficence
Non-Maleficence
Fidelity
Veracity
Justice

SIX BIOETHICAL PRINCIPLES



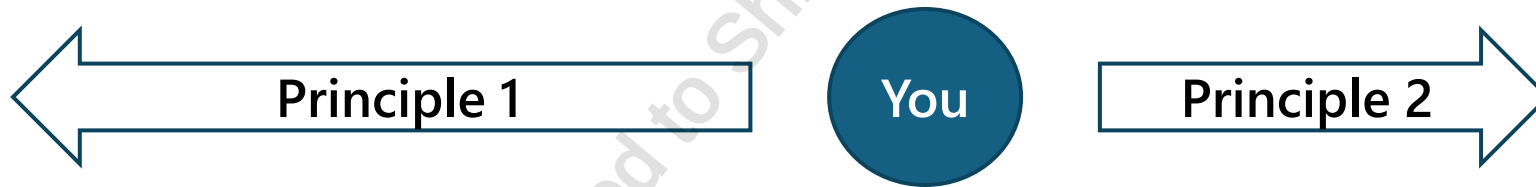
No principle routinely trumps the others!

1. **BENEFICENCE**: Being of **benefit** to the patient by (promoting the **patient's best interests** and well-being)
2. **NONMALEFICENCE** (3 duties):
 - a. **Do no unjustified harm** by **refraining from actions (or inactions)** that may harm patients
 - b. **Prevent** harm; and
 - c. **Remove** harm
3. **JUSTICE**: **Fairness** in the distribution of resources, treatments and harms
4. **AUTONOMY**: Respecting the **patient's free will** and decisions to **choose/refuse treatment**
5. **HONESTY/VERACITY**: Telling the **truth** without deception or an intent to mislead
6. **FIDELITY**: **Loyalty** and **keeping our promises** to the patient. This forms the ethical basis of the practice of **confidentiality** (the act of keeping a patient's information private) and keeping yourself **free of conflicts of interest**.

Don't participate in a cover-up! If you have good reason to suspect something illegal or unethical is going on, let the authorities know.

WHAT IS A MORAL DILEMMA?

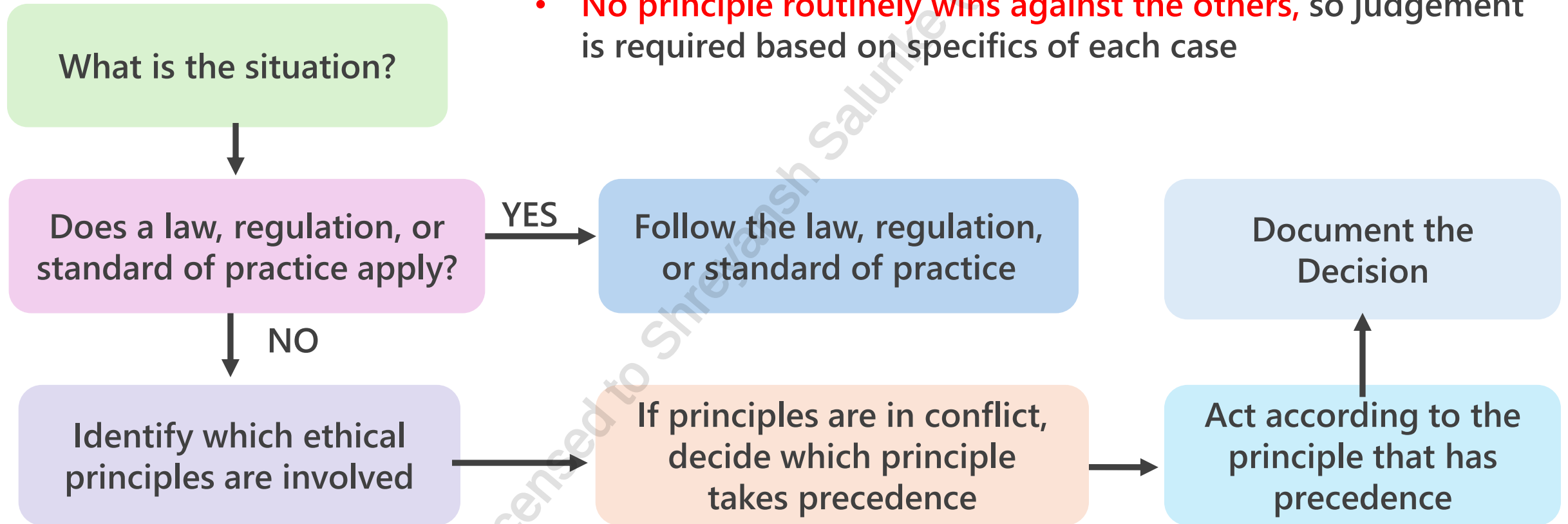
- A situation in which you must choose between 2 or more alternative actions
- Each action is based on different moral principle, making it impossible to act in such a way and follow all the moral principles
- No matter what you do at least one principle will be violated



PHARMACHIEVE'S ETHICAL DECISION-MAKING FRAMEWORK

When **>1 principle** applies to a situation, which one wins?

- **No principle routinely wins against the others**, so judgement is required based on specifics of each case



AUTONOMY

Patients' right to make their own decisions

If a patient has the capacity to make decisions, you should defer to the patient's wishes

Exceptions: patients who lack capacity and emergencies (e.g. when patient is unconscious and you cannot get patient's consent).

Example of upholding autonomy:

You follow the **informed consent** protocol by explaining in detail the treatment alternatives, the relative benefits and risks of each treatment, while also providing the patient the choice of choosing among the options

Example of violating autonomy:

You choose a treatment for the patient when there are many options without explaining the alternatives because "you know what's best"

Common Dilemmas:

- Beneficence vs. Autonomy

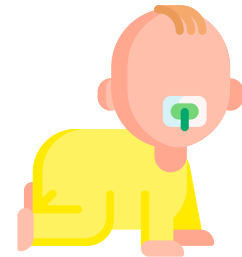
PATERNALISM

NOT an ethical principle; it is **opposite of autonomy** and occurs when a healthcare professional thinks their will should override the patient's

Occurs because the healthcare professional (i) believes that they know the patient's best interests better than the patient and (ii) acts on this belief in a way that diminishes patient's autonomy. Often beneficence and non-maleficence are used by healthcare professionals to justify paternalism.

Examples:

1. A pharmacist does not disclose the side effects of a drug that the patient needs for their care because she thinks the patient will not take the needed drug if the patient knows what the side effects are.
2. When a patient asks how to treat a condition, a pharmacist only discusses the one treatment she thinks is best. She does not counsel on the risks and benefits of that treatment or the risks and benefits of the alternatives.
3. A patient refuses a blood transfusion on religious grounds, but a healthcare professional knowingly transfuses blood anyway with the reason that "it will save the patient's life."



NONMALEFICENCE

1. Do no **needless** harm (physical, emotional, psychological) through your actions/inactions
2. Prevent harm
3. Remove harm

Examples of upholding nonmaleficence:

1. You recommend against an addictive pain killer because an alternative non-addictive pain killer with fewer side effects exists (thus **preventing** harm)
2. You institute TALLman lettering to prevent similar sounding medications from mistakenly being interchanged (thus **preventing** harm).
3. You do not recommend treatments where the risks outweigh the benefits (thus **doing no harm**)
4. You discontinue unneeded medications (thus **removing** harm)

Example of **harm from actions**:

4. You make an error while dispensing
5. You recommend a non-evidence-based treatment
6. You prescribe a potentially addictive pain medication when a just-as-effective non-addictive one exists.

Example of **harm from inactions**:

7. Forgetting to counsel a patient about how to avoid the side effects of a drug (e.g. rinse mouth after using corticosteroid spray) that leads to patient developing thrush

Common Dilemmas:

- Beneficence vs. Non-maleficence
 - Balancing benefits and risks of treatment

BENEFICENCE

Being of benefit to the patient. Doing what is best for the patient

Examples:

- Providing vaccinations for the general population
- Encouraging a patient to quit smoking and start an exercise program
- Talking to the community about Sexually Transmitted Infections (STI) prevention
- Recommending therapeutic options to a patient
- Dispensing a medication
- Counselling a patient
- Administering a medication

Common Dilemmas:

- Beneficence vs. Non-Maleficence
- Beneficence vs. Autonomy
 - Patient's autonomous decision conflicts with the pharmacist's duty to look out for the patient's best interests

PRACTICE CASE

You are a manager of a pharmacy with only 1 other pharmacist and no other staff. You are certain that you and your pharmacist colleague are the **ONLY** ones with access to narcotics, but a large volume of narcotics has repeatedly and persistently gone missing, and you have nothing to do with it. What should you do?

- a) Talk to the colleague about the situation
- b) Install a video camera
- c) Report your suspicions to your pharmacy regulatory authority
- d) Do nothing

JUSTICE

Fairness in the distribution of benefits, harms, and resources

How do you know what's equitable? John Rawls suggests you imagine yourself behind a **veil of ignorance** where you don't know who you are, where you were born, your natural abilities, your position in society, wealth, sex, race, nationality, or individual tastes, etc. so you don't know who your decision will benefit, including you. How would you organize society?

Examples whose analysis require considering the principle of justice:

1. For a clinical trial of a drug, you recruit people of low income because it is easier to recruit them than wealthy people.
 - What principle is violated by this policy and why? What action(s) should you take to fix it?
 - Would your answers change if the study caused benefit vs. harm?
2. An expensive drug is only made available to wealthy people who can afford it.
 - What principle is violated by this policy and why? What action(s) should you take to fix it?
 - Which principle did you violate?
3. You are interviewing someone for a pharmacy technician job. What questions can you ask during the interview? (a) religion (b) age (c) criminal record (d) ethnicity.
4. In the midst of the COVID-19 outbreak, you hear that Remdesivir might help infected people fight the disease. The drug is in short supply, but you stockpile the drug "in case your family needs it," even though your friends and family aren't at high risk or infected. What principles apply to this case?

FIDELITY

Loyalty and honouring one's commitments to the patient

- This duty of loyalty is the basis of trust in the healthcare provider
- One way we remain loyal is by **avoiding conflicts** of interest, **keeping our promises** and **keeping patient information confidential**. This duty is not only an ethical one but is also required by law

Examples:

You promised to meet a patient (P1) at the time of discharge to go over their medications. But another patient (P2) was just admitted who requires your urgent attention. You cannot make both appointments at the same time. You decide to deal with the admission for P2 because the discharge for P1 can be delayed. What principle was upheld, which was violated and to whom?

You work in a pharmacy near a sexually transmitted infections (STI) clinic. A patient's sibling asks if her brother is a patient? What do you tell her?

FIDELITY

Continued

A pharmacist on the Medical Advisory Committee (MAC) is meeting with other committee members to decide which drugs to include in the hospital formulary. The drug being discussed is one that the pharmacist was paid (by the drug company) to provide talk on at a conference. What should the pharmacist do?

Implied Consent:

- Sometimes a patient does not have to give you permission to reveal confidential information. Examples include sharing health information with other healthcare providers in the **circle of care** and responding to emergencies

A patient receives an A1C test at a pharmacy; the result is very high. What actions should the pharmacist take?

A pharmacist adjusts the dose of a medication. What actions should the pharmacist take with respect to other healthcare professionals in the circle of care?

You believe a mentally unstable person will imminently harm a person or a group of persons. This includes the person possibly harming themselves (e.g. suicide). What action should you take?

VERACITY

Duty to tell the whole truth and not mislead

Examples:

- You use placebo medication for addiction withdrawal without telling the patient because your research indicated this was the best way to taper the patient off the drug. Which principle was upheld? Which was violated?
- A physician asks you not to tell a patient about the side-effects of a drug that she believes could significantly help the patient. The physician fears that if you tell the patient about the possible side effects, the patient might not accept the medication. Which principal was upheld? Which principle was violated?
- A pharmacist dispenses 50mg of a drug instead of 5mg. The patient takes it, but suffers no harm. What is the pharmacist's next step?

ETHICAL DILEMMAS

Practice Cases

1. You are about to close the pharmacy and a customer arrives at the last minute looking to refill a prescription for acute gout pain. If you refuse to dispense, which principle is violated?
2. You were very busy in the pharmacy and forgot to counsel a patient on a new prescription.
 - a) Which ethical principle was breached?
 - b) What if the patient experiences an adverse effect that your counselling could have prevented?
3. A pharmacy owner has decided to stop selling subcutaneous syringes to reduce customer traffic of suspected drug addicts at night. What ethical principle is breached?
4. You are concerned about the OTC sales of several products containing pseudoephedrine (because of its potential for abuse in being transformed into crystal methamphetamine) or to increase the blood pressure of patients with hypertension. As a pharmacy owner, you decide to keep this product behind the counter so that you can counsel and/or restrict access.
 - a) Which ethical principle(s) are involved?
 - b) Which principle may be violated depending on how the pharmacist interacts with a patient?

CAPACITY VS. COMPETENCE

There is a trend in society and the courts towards autonomy for patients that have the capacity to make decisions

	Capacity	Competence
Type of concept	Ethical	Legal
Who makes the assessment?	Health Care Professionals treating the patient (pharmacist, MD, etc.)	Judge
How long does it take for a determination to be made?	Immediate/brief	Time consuming legal process
How clear is its existence?	Often unclear. Has to be estimated by the Health Care Professionals	Very clear. A person either has competence or they do not.
Is it temporary or permanent? Does it apply to some decisions or all decisions?	Can temporarily fluctuate and is decision/task specific	Is permanent (until changed by a court) and applies to all decisions in most cases
If it doesn't exist, who makes the decision in place of the patient?	See chart on next slide	Court appointed Guardian

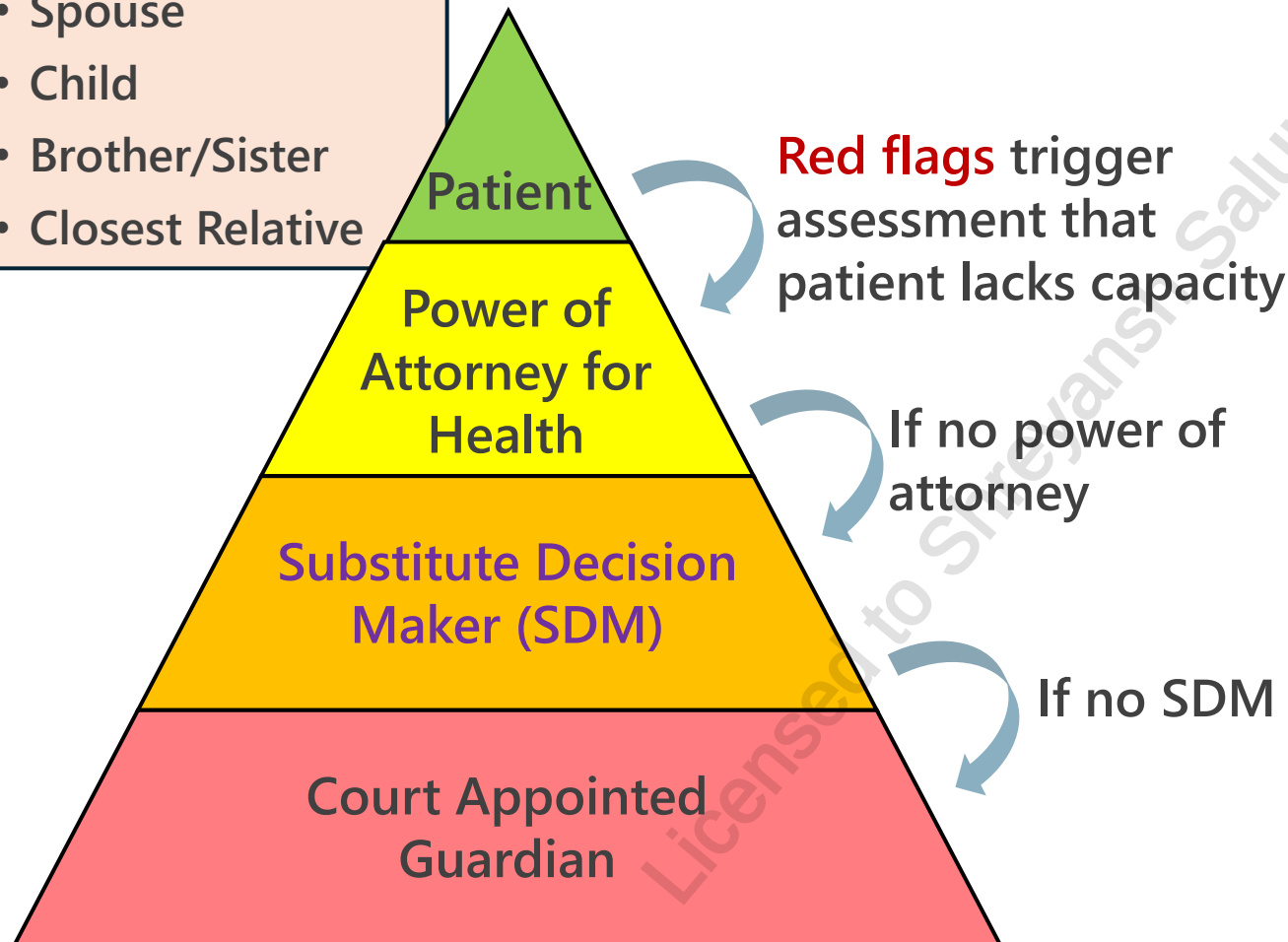
Source: Adapted from Phil Lawson MD, *Ethical Challenges in Medical Decision Making*, 2012

WHO MAKES A HEALTHCARE DECISION?

It depends. Assume patient has capacity. But adjust your assessment accordingly

Hierarchy for SDM
(varies by province)

- Spouse
- Child
- Brother/Sister
- Closest Relative



When to assess for capacity (red flags):

- Significant mental illness, dementia, delirium or disability
- An inability to understand information or decisions
- Either end of age spectrum
- Polypharmacy (linked to cognitive impairment in the elderly)
- Making choices inconsistent with prior values/choices
- Refusal of a low-risk, high-benefit treatment
- Severe depression

CONSENT AND CAPACITY

In order to be able to give consent, a person needs the capacity to make decisions

In the previous slide we discussed *WHO* makes a healthcare decision. In this slide we discuss *HOW* these people can make a healthcare decision for a patient who loses capacity. There are generally 3 approaches. They are ordered from most desirable to least desirable to maximize patient autonomy:

1. **Living Will** (also known as **advance directives**): Written document stating the patient's wishes to healthcare professionals in case they lose the capacity to make decisions.
2. **Substituted Judgement**: If no living will exists, or if the living will does not cover the situation at hand, the "Substituted Judgement Approach" is generally used. Under this approach surrogates, usually family members try to determine what the patient would have decided if they still had the capacity to make decisions.
3. **Best Interests**: If the patient left little or no indication on what they would have decided, the surrogates decide based on what would be good for the patient. The moral principle underlying this approach is beneficence.

INFORMED CONSENT

“[The] process by which the treating health care provider discloses appropriate information to a competent patient so that the patient may make a voluntary choice to accept or refuse treatment” (Appelbaum, 2007)

Informed consent recognizes that **patients can't make informed decisions without being given the necessary facts.** **If you don't fully inform the patient, you violate their autonomy.**

Informed consent **requires the patient to know:**

- ✓ The nature of the decision/ procedure
- ✓ Reasonable alternatives to the proposed intervention
- ✓ Relevant risks, benefits and uncertainties related to each alternative
- ✓ Assessment of patient understanding
- ✓ Acceptance of the intervention by the patient

INFORMED CONSENT

Practice Cases

1. There are a number of possible treatment options for a patient, each with its own significant pros and cons, but you only tell the patient about one treatment you prefer and do not tell them that alternatives exist.
 - Does this provide the patient the opportunity to give informed consent?
 - Can the patient exercise autonomy without knowing the alternatives?
2. You get a verbal prescription for dabigatran. You are told by the physician to ensure that no print-out is given about the drug to the patient because the bleeding risk may frighten the patient from taking the medication.
 - Which ethical principles are involved?
 - What is the appropriate action to take?

ETHICS BOARDS

- Generally, **research involving human subjects requires prior approval** by a research ethics board (REB).
- REB usually **consists of clinicians** (doctors, nurses, pharmacists), **a legal member**, and **≥1 member(s) of the public** (to reflect the interests of the sick)
- Research proposals need to be reviewed to make sure they are **consistent with ethical principles**. Some of the things REBs consider when reviewing a proposal:
 - ✓ Respect for human dignity
 - ✓ Informed consent
 - ✓ Privacy and confidentiality
 - ✓ Justice
 - ✓ Balancing harms and benefits

REFUSAL TO FILL For moral or religious reasons

A pharmacist is permitted to decline providing certain products/services but must ensure an **alternate source** is arranged for those products/services. When doing so, the pharmacist should **avoid sharing their moral objections**, and should arrange the alternative care **in a way that minimizes the suffering of the patient**.

ETHICS OF PRESCRIBING Self and Family Member Prescribing

A personal, non-professional relationship makes it **inappropriate** to “treat” an individual with objectivity (includes ordering and performing tests, making/communicating a diagnosis, and prescribing)

- **Pharmacists and physicians should NOT treat either themselves or family members except:**
 - For “**minor conditions**” and “**emergency situations**” where no other qualified health care provider is readily available
 - If care is provided, then this should be transferred to another qualified healthcare professional as soon as possible

EXAMPLE OF AN ETHICAL ISSUE IN THE NEWS



CBC News Article

“Quebec woman speaks out after pharmacist denies her morning-after pill due to his religious beliefs”



Link to article

PRACTICE CASE

A pharmacist is working alone at a pharmacy, when a young lady presents a prescription for Mifegymiso® (Mifepristone/Misoprostol), which is indicated for medical abortion. The pharmacist has conscientious moral objections to dispensing Misoprostol. What is the pharmacist's best and most appropriate action to take:

- a) Tell the patient that they cannot fill the patient's prescription based on their religious beliefs.
- b) Tell the patient they will be unable to fill the patient's prescription but show the patient the location of a pharmacy in close proximity that can fill the prescription
- c) Fill the patient's prescription despite conflict with their moral beliefs
- d) Refer the patient to a walk-in clinic for a medical assessment

ETHICS OF PRESCRIBING

Self and Family Member Prescribing

- Physicians should **never prescribe narcotics, controlled drugs, psychotropic drugs or any drugs with addictive properties** for either themselves or family members
 - Non-compliance with the above can be subject to allegations of professional misconduct
- Prescribing for family members is not banned by law but it is banned in some but not all provinces through their respective code of ethics (e.g. for pharmacists, physicians, dentists, etc.). It is not illegal, but it can be unethical
 - Regardless, a **pharmacist can refuse to dispense if they think abuse is occurring**

Case: You are pharmacist at a community hospital in a province where pharmacists can prescribe a certain medication. You notice a colleague consistently prescribing for themselves and family members. What should you do?

- a) Talk to the colleague about the situation
- b) Tell the family members they should get their prescriptions from someone else
- c) Report the activity to the College

CASE 1

You work at a sexually transmitted infections (STI) clinic. You call a patient's home phone to confirm an appointment but someone else answers the phone. You leave a message.

Which principle was violated?

CASE 2

A patient with hypertension arrives at your pharmacy to ask about the side effects of pseudoephedrine because they have nasal congestion that affects their quality of life. You tell them that pseudoephedrine can increase blood pressure and that they should discontinue the medication, but you have not recommended an alternative therapy.

What is the most appropriate action to take?

- a) Refuse the sale and tell the patient to consult their physician
- b) Sell the product but tell the patient to closely monitor their blood pressure
- c) Sell the product because the patient has the right to choose their therapy
- d) Recommend a localized alternative like a saline spray or topical decongestant

CASE 3

A physician prescribes an antibiotic, in response to the patient's request for one to treat the influenza virus.

Which principal has been upheld and which has been violated?

CASE 4

A patient asks you to fill a prescription for Mifegymiso® (Mifepristone/Misoprostol) indicated for medical abortion.

If you are not feeling comfortable, can you refuse to fill this prescription?

ETHICS OF RELATIONSHIPS

Pharmacists and Pharmaceutical Industry

Guidelines have been developed by a number of associations with common elements:

- Patients should be the pharmacist's primary concern in all aspects of pharmacy practice, including interactions with industry
- Pharmacists **should not** solicit or accept gifts from industry that might influence or appear to influence objectivity, independence, or fairness in clinical and professional judgment (but gifts of minimal value (<\$100) e.g. pens are acceptable)
- Pharmacists should disclose financial, consulting, or other relationships that are or appear to constitute conflicts of interest
- Pharmacists who make decisions regarding the purchase, prescribing, or use of drugs or devices by others through formulary committee deliberations, clinical practice guideline development, or administrative responsibilities **should avoid** financial, consulting, or other relationships with industry that are or appear to constitute conflicts of interest
- Pharmacists who are members of an ethics review board **should avoid** any real, potential, or perceived conflicts of interest that could occur in connection with matters before the board
- Pharmacists who participate in industry associated research **should only do so if that research meets accepted ethical, regulatory, and scientific standards**

PRACTICE CASE

A pharmacist offers to pay a group of physicians to co-locate their clinic in a common building with a shared lobby with his pharmacy. The pharmacist is considering offering free rent to the physicians. What action reduces the most significant conflict of interest?

- a) Offering the physicians market rent
- b) Asking the physicians to tell their patients that they can get their prescriptions filled at any pharmacy
- c) Telling the physician that there is no expectation for them to direct patients to the pharmacy or to prescribe a certain volume of prescriptions

AVOIDING ACTUAL OR PERCEIVED CONFLICT OF INTEREST IN BUSINESS DEALINGS

From the Ontario College of Pharmacists

Source: Pharmacy Connection, Ontario College of Pharmacists, 2017

SUMMARY OF THE ISSUE

This issue occurred when a pharmacist placed an advertisement for physicians to work in the medical clinic adjacent to the pharmacy in a building owned by the pharmacist. The advertisement offered "zero overhead" for the physician and payment of expenses by the pharmacy.

A physician sought more information from the pharmacist who informed the physician that the unoccupied space did not have existing expenses, but clarified that any new costs incurred by the clinic would not be covered by the pharmacy. The pharmacist expressed that there was no obligation or expectation of the physician occupying the clinic to encourage patients to fill their prescriptions at the pharmacy, and

conflict of interest regulations for both pharmacists and physicians.

WHY DID THIS HAPPEN

The main concern here is conflict of interest, whether actual or perceived. It could be perceived that offering clinic space owned by a pharmacist at no cost would be an inducement to prescribers, regardless of whether the prescriber is actually referring patients or suggesting they have their prescriptions filled at the pharmacy. It could be perceived that such an arrangement would encourage a prescriber to engage in such activity.

It could also be perceived that a pharmacist's professional judgment might be compromised in situations in which a pharmacist was receiving increased business

pharmacist relationship and thus the pharmacy's business.

Pharmacy professionals must avoid situations that are, or may reasonably be perceived to be, a conflict of interest. It is also expected that pharmacy professionals will avoid dual relationships and other situations which may present a conflict of interest and potentially affect their ability to be impartial and unbiased in decision-making.

COMPLAINT OUTCOME

The College's Inquiries, Complaints & Reports Committee (ICRC) oversees investigations of each complaint or report the College receives. The Committee considers a pharmacy professional's conduct, competence and capacity by assessing the facts of each case.

PHARMACIST CODE OF ETHICS

- A pharmacist should use their knowledge, skills and resources to:
 - ✓ Provide care to patients
 - ✓ Contribute to societal needs
 - ✓ Act as ambassadors (or stewards) for their profession
- This means that the pharmacist needs to :
 - ✓ Maintain professional competence
 - ✓ Develop professional knowledge & skills through life-long learning
 - ✓ Maintain good relationship with others (respect others)
 - ✓ Make patient care their first concern
 - ✓ Exercise professional judgment in the interest of patients & public
 - ✓ Take responsibility for their work & practices
 - ✓ Encourage patients to participate in decisions about their care

EXAM TIPS

- **If Law vs Ethics, the Law wins:** If a provincial or federal law exists (e.g., mandatory reporting, narcotic regulations), it always trumps ethical principles.
- **Protect the Patient First:** Never choose an answer that prioritizes pharmacist convenience, pharmacy revenue, or saving a prescriber's ego over patient safety.
- **Do Not "Pass the Buck":** Avoid answers where the pharmacist blindly says, "Go ask your doctor" without taking professional responsibility first. Pharmacists are independent practitioners who must attempt to resolve issues within their scope.
- **Eliminate, Don't Mitigate:** In conflict of interest questions, the best answer removes the conflict entirely rather than just "managing" it.
- **Assess Before Acting:** If an option suggests "gathering more information," "asking open-ended questions," or "interviewing the patient," it is almost always the correct first step. Do not make clinical assumptions.

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CHANGE LOG

August 2025

- Updated definitions of morals and ethics on slide 4
- Updated conflict of interest case on slide 34
- Added new slide with OCP reference on slide 35

November 2025

- Minor edits (no reprinting necessary)
- Updated slide 27 (changes in red)

March 2026

- Added 3rd case to slide 17
- Changed nature of question on slide 23 from identifying principle to identifying most appropriate action
- Added “exam tips” (now slide 37)
- Added answer choices to slide 30